

Emergency management of ADRENAL CRISIS

NICE 2024, NG243



Red Whale

GEMS

Guidelines & Evidence Made Simple

*** NICE guidance on emergency management applies to those $\geq 16y$ ***

For those $<16y$, NICE recommends using the British Society of Paediatric Endocrinology and Diabetes (BSPED) Consensus Guidelines on Adrenal Insufficiency (<https://www.bsped.org.uk/adrenal-insufficiency>). The central tenets of treatment (hydrocortisone, rapid transport to hospital and fluids) all apply, but doses depend on age/weight.

Recognising an adrenal crisis

Consider adrenal crisis as cause if:

Critically unwell with any of:

- Low blood pressure (including postural hypotension)
- Hyperpigmentation
- Hyponatraemia
- Hypoglycaemia (particularly in children)
- Circulatory shock or collapse
- Condition failing to respond to initial treatments

or

Unwell with milder symptoms BUT known adrenal insufficiency OR at high risk of adrenal insufficiency

- Lethargy
- Pallor
- Clamminess
- Feeling cold/feverish
- Confusion/altered mental state
- Weakness

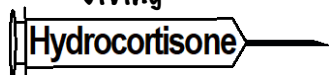
In primary care, this is likely to be the more common presentation.

Have you flagged the notes of all those with known adrenal insufficiency?

Emergency management of adrenal crisis (in those $\geq 16y$)

The 3 essential ingredients in the management of adrenal crisis:

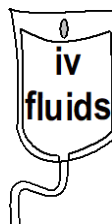
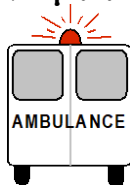
Giving



100mg intravenously or intramuscularly

Anyone can give it intramuscularly: patients, relatives and carers should be trained to do this

Rapid transport to hospital



Ideally within 30 minutes
(NICE implies this is given in hospital setting)
1L of 0.9% sodium chloride
by iv infusion over 30mins

Once in hospital

- Ensure frequent monitoring of BP, heart rate, electrolytes, glucose.
- Hydrocortisone by iv infusion over 24h (or im/iv injections 4x daily) until haemodynamically stable and able to absorb oral glucocorticoids. Patient will be switched to oral drugs when possible (at higher than usual dose – often using doses specified in sick day rules) until cause resolved and person is clinically stable.
- Continue to give iv fluids (0.9% sodium chloride) until haemodynamically stable (rate determined by haemodynamic parameters and electrolyte state).
- Identify and treat underlying cause.
- Seek advice from endocrinologists throughout admission.

Prevention is better than treatment!

Patients need to be confident in using the sick day rules: this should reduce the need to use emergency kits.

We make every effort to ensure the information in these pages is accurate and correct at the date of publication, but it is of necessity of a brief and general nature. The information presented herein should not replace your own good clinical judgement, or be regarded as a substitute for taking professional advice in appropriate circumstances. In particular, we suggest you carefully consider the specific facts, circumstances and medical history of any patient, and recommendations of the relevant regulatory authorities. We also suggest that you check drug doses, potential side-effects and interactions with the British National Formulary. Save insofar as any such liability cannot be excluded at law, we do not accept any liability for loss of any type caused by reliance on the information in these pages. December 2024. For full references see the relevant Red Whale articles.